

Hysterectomy for womb cancer

A hysterectomy is the main surgery for womb cancer.

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Surgery for womb cancer

Surgery is the main treatment for [womb cancer](#). The surgery is done by a gynaecological surgeon with experience treating gynaecological cancers.

After surgery, the surgeon will be able to tell you more about the [type](#), [stage and grade](#) of the cancer. It can take about 2 to 3 weeks to get the test results back.

Surgery to remove the womb

Surgery to remove the [womb](#) is called a hysterectomy.

During surgery for womb cancer, the surgeon usually removes:

- the womb and the cervix (a total hysterectomy)
- the fallopian tubes and both ovaries (a bilateral salpingo-oophorectomy or BSO).

An operation to remove all of the organs listed above is called a total hysterectomy and bilateral salpingo-oophorectomy (BSO).

Your surgeon may want to check for cancer cells in other places. This will depend on the:

- [type](#) of womb cancer
- [stage](#) of the cancer
- the [grade](#) of the cancer cells.

Your surgeon might talk to you about removing:

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[lymph nodes](#) close to the womb (pelvic nodes)

- lymph nodes higher up in the abdomen (para-aortic nodes)
- the omentum – a layer of fat and tissue covering the organs in the front of the abdomen (tummy area).

Your surgeon and specialist nurse will explain the benefits and disadvantages of surgery. They will talk to you about what to expect after surgery and the possible side effects.

Areas removed during a total hysterectomy and bilateral salpingo-oophorectomy

Image: The illustration shows a front view of a female body from above the belly button to the top of the thighs. This illustrates the female reproductive system. At the top of the section shows two thin, curved tubes. These are the fallopian tubes. At the end of each fallopian tube is an ovary, shown as a small blue oval shape. The womb is positioned in the middle of the pelvis and has a pear-like shape. It is wider at the top and narrows toward the bottom. Down from the uterus is the cervix, a narrow, neck-like structure that leads into the vagina, which continues vertically toward the lower edge of the image. The vulva is indicated at the external opening at the bottom. There is black box made up of dotted lines, This goes around the fallopian tubes, ovaries, womb and cervix, but stops before the vagina and vulva underneath. This shows the areas that are removed during a total hysterectomy and bilateral salpingo-oophorectomy.

Side view of female organs

Image: The illustration shows a side view of a female body from below the chest down to the upper thigh. It shows the structures and organs in the abdomen and pelvis. There are 3 small openings into the pelvis between the legs. The opening closest to the back of the body is called the anus. It leads up into a tube-shaped space called the back passage or rectum. The opening closest to the front of the body leads up through a short narrow tube called the urethra. This leads into a rounded hollow organ called the bladder. The opening in the middle leads through a longer tube called the vagina. This leads into a hollow organ with thick walls called the womb or uterus. The lowest part of the womb is highlighted where it meets the vagina. The highlighted area is called the cervix. The womb sits behind and over the bladder. Above the bladder and womb, a large space is shown. This space fills the front half of the body. Most of the space contains an irregular bundle of thick tubing. This is the bowel. Above the bowel is a round hollow organ called the stomach. Above the stomach is a large solid organ called the liver. A thin lining is shown covering the walls of the space. The lining also covers the surfaces of the liver, stomach, bowel, bladder and womb. This lining is the peritoneum. A long flap of tissue is shown hanging down in the space in front of the bowel. This is the omentum. A line of connecting bones are shown following the curve of the back down. These are the spine. The bones come to a point behind the rectum.

Early menopause

If you still have periods, having a total hysterectomy and BSO will cause your periods to stop. This is called menopause. This can cause [symptoms](#) such as hot flushes and vaginal dryness.

If the cancer is at an [early stage](#) and the cancer cells are low grade, it may be possible to not remove the ovaries. This can stop early menopause and menopausal symptoms.

Before surgery for womb cancer

As well as information about your surgery, your surgeon or specialist nurse may suggest things you can do to improve your general health before and after surgery. This is sometimes called [prehabilitation](#).

Prehabilitation can help you to:

- improve your general fitness and nutrition before surgery
- reduce anxiety
- get information and guidance about [pelvic floor exercises](#) to help recovery before and after surgery to the pelvic (lower tummy) area.

It can help:

- reduce your time in hospital
- reduce the risk of complications from surgery
- give you a feeling of taking control of health and [recovery](#).

Some hospitals might have a prehabilitation programme or team. Or it may be support from your specialist nurse and other professionals in the MDT who plan your care.

Pre-operative assessment

You will have a [pre-operative assessment](#) around 1 or 2 weeks before your surgery. At this appointment, you will meet with a nurse who will do tests to check you are well enough for surgery. You will have blood tests and may have an electrocardiogram (ECG) to check your heart. You will also get information about your surgery and how to prepare.

Stopping smoking

If you smoke, [stopping](#) at least 2 weeks before your surgery will help reduce the risk of chest problems. It will also help:

- your wound to heal [after the operation](#)
- reduce side effects of further treatment, such as [radiotherapy](#).

Your GP can give you advice and support.

Enhanced recovery

Your surgeon and specialist nurse may also talk about what you can do to reduce the time you spend in hospital and help you recover as quickly as possible. This is called enhanced recovery. It also aims to get you actively involved in your recovery.

Some hospitals have an enhanced recovery after surgery (ERAS) programme.

Your doctor or nurse will tell you if an ERAS programme is available and suitable for you. This may include:

-

eating and drinking as close as possible before surgery, and as early as possible after surgery, to reduce the time you go without nutrition

- types of surgery such as vaginal, keyhole or robotic surgery which can be quicker to recover from than abdominal (open) surgery
- good pain management so you can get out of bed and move as early as possible after surgery
- getting out of the hospital gown and dressed into daytime clothes as soon as possible after surgery.

Types of hysterectomy

A hysterectomy can be done in different ways, depending on the [type](#) and [stage](#) of cancer. Your surgeon will talk to you about the best type of surgery for you.

Keyhole (laparoscopic) surgery

Keyhole (laparoscopic) surgery is when the surgeon makes several small cuts in the tummy (abdomen) instead of a large single cut. This leaves small wounds, so you usually recover faster. The surgeon puts a thin tube with a light and camera on the end through one of the small cuts in the skin. This tube is called a laparoscope. It allows the surgeon to examine the area and do the surgery. The camera shows an image of inside your body on a screen. Air (gas) is pumped into your abdomen or pelvis to help get a better view and separate parts of the body. Other instruments needed to do the surgery go through the small cuts.

Keyhole robotic surgery

Keyhole robotic surgery is when keyhole surgery is helped by a machine. Instead of the surgeon holding the laparoscope and surgical equipment, they are attached to robotic arms. The surgeon controls the robotic arms, which can move very precisely and accurately.

Abdominal (open) surgery

Abdominal (open) surgery is when the surgeon makes one cut in the tummy (abdomen). After this surgery, you have a wound that goes across your tummy just above the hips, or that goes down from the belly button to just above the pubic hair.

Vaginal surgery

Vaginal surgery is when the surgeon operates through a cut at the top of the vagina. The surgeon may combine this with laparoscopic surgery.

What else may be done during the surgery

During surgery, the surgeon will check nearby lymph nodes and other organs to find out more about the [stage](#) of the cancer.

Removing lymph nodes

The surgeon may remove [lymph nodes](#) to check for cancer cells. These may be nodes close to the womb (pelvic nodes) or higher up in the abdomen (para-aortic nodes). This may not be needed if you have a small cancer with low grade cancer cells.

Sometimes the surgeon might test the lymph nodes with special dye during surgery. After this, they might only remove the 1 or 2 lymph nodes closest to the cancer. This is called a sentinel lymph node biopsy. If these nodes contain cancer cells, more treatment might be needed after surgery.

If pre-operative scans show the cancer may have spread, they may remove more lymph nodes during the surgery.

Your surgeon will talk to you about the possible benefits and disadvantages of removing lymph nodes.

Surgery if the cancer has spread outside the womb

You may have surgery to remove as much of the cancer as possible if the cancer has spread to organs such as the:

- peritoneum
- omentum
- bladder
- bowel.

Sometimes [chemotherapy](#) is given before this type of surgery to help shrink and control the cancer.

If the cancer has spread through the pelvis, sometimes a major operation can be done to remove the bladder and bowel as well as the womb. This surgery is called a [pelvic exenteration](#).

If the cancer has spread to the liver or lungs, surgery is not usually possible. Rarely, surgery can be done to remove a secondary cancer. But this is only helpful if the cancer is in one area and there are no signs of cancer anywhere else.

These types of surgery can help control the cancer and help make other treatments more effective.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our womb cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.



Womb cancer forum

An anonymous network of people affected by womb cancer which is free to join. Share experiences, ask questions and talk to people who understand.

Get help